

Gynaecological surgeries

`The art of surgery consists in knowing when not to operate.` Sir John Ericksen

Objectives

By the end of this session, students should be able to :

- ❖ Understand indications for common gynecological surgeries
- ❖ Understand minimally invasive vs. open approaches.



General surgical principles

- Indication for surgery and decision making
- Pre-assessment
- Consent
- Surgical safety
- Good surgical practice
- Post-operative care and recovery

Key learning points

- Consent for surgery is a continuous process and should be fully documented.
- All alternative options and potential complications should be explained to the patient in a format and language they can understand
- Where possible, site-or surgeon-specific figures for success and complications should be used
- Mental capacity is required to give consent
- Assessing mental capacity when doubt exists is a formal process and must be documented
- When capacity is lacking treatment decisions can be made in patients best interests.

Classification of gynecological operations :

1.Diagnostic procedures:

- Colposcopy
- Hysteroscopy
- Diagnostic laparoscopy

2.Operative procedures

- Hysterectomy
- Myomectomy
- Ovarian cystectomy
- Tubal ligation
- D&C
- Pelvic floor repair

Hysterectomy

Types:

- Total hysterectomy
- Subtotal hysterectomy
- Radical hysterectomy
- With or without salpingo-oophorectomy

Routes:

- Abdominal
- Vaginal
- Laparoscopic
- Robotic

Indications for Hysterectomy :

- Uterine fibroids (symptomatic)
- Abnormal uterine bleeding not responsive to treatment
- Endometriosis/adenomyosis
- Pelvic organ prolapse
- Gynecological malignancies

Complications of hysterectomy:

Intraoperative:

- Hemorrhage
- Bladder/ureteric injury
- Bowel injury (Rectal injury)

Postoperative:

- Immediate onset of menopausal symptoms (if ovaries are removed in a premenopausal person)
- Wound infection (common) , pelvic infection (uncommon)
- Secondary hemorrhage
- Vesicovaginal or rectovaginal fistula
- DVT/PE
- Vaginal prolapse (late)
- New bladder symptoms(late) both overactive bladder and stress incontinence

Myomectomy:

Is removal of uterine fibroids while preserving the uterus

Approaches:

- Abdominal (laparotomy)
- Laparoscopic
- Hysteroscopic

Indications :

- Symptomatic fibroids
- Infertility due to submucosal fibroids
- Recurrent pregnancy loss
- Big fibroids causing bulk symptoms

Complications

- Bleeding
- Adhesion formation
- Uterine rupture in future pregnancies (rare)
- Needing for hysterectomy (if uncontrolled bleeding)

Ovarian cystectomy

Is removing of the ovarian cyst while preserving the ovarian tissue

Indications:

- Persistent simple ovarian cyst
- Endometriomas
- Dermoid cysts
- Torsion (emergency)

Laparoscopy in gynecology: safe entry techniques are essential

Can be used for:

- Suspected ectopic pregnancy
- Ovarian cyst accident and acute pelvic pain
- Undiagnosed pelvic pain
- Tubal patency testing
- Sterilization

Benefits:

- Less pain.
- Shorter hospital stay.
- Faster recovery.

Complication:

Port site infection and hernia
Damage to any intra-abdominal
Structure

Hysteroscopy:

Diagnostic and operative uses:

- Any abnormal bleeding from the uterus :
- Post menopausal bleeding , irregular menstruation, intermenstrual bleeding , post-coital bleeding , persistent heavy menstrual bleeding
- Suspected uterine malformations
- Suspected Asher an syndrome
- Persistent discharge

Advantages:

- Minimally invasive.
- Quick recovery
- Direct visualization.

Complications:

Perforation of the uterus
Cervical damage if dilatation done
Ascending infection

Pelvic floor surgeries

Includes:

- Anterior/posterior colporrhaphy
- Sacrospinous fixation
- Sacrocolpopexy

Indication :

Pelvic organ prolapse

Gynecological cancer surgeries:

May include :

- Radical hysterectomy (cytoreductive surgery)
- Debulking surgeries

Enhanced Recovery After Surgery

Pre-operative:

- Counseling
- Optimization of health
- Avoid overnight fasting and dehydration
- Carbohydrate loading

Perioperative :

- VTE thromboembolism prophylaxis
- Minimally invasive surgery where possible
- Antibiotic prophylaxis (certain procedures only)
- Prevention of hypothermia
- Avoid unnecessary tubes or drains
- Short acting anesthetics

Post-operative:

- Good analgesia (avoid opioids)
- Early mobilization
- Early resumption of fluids and normal diet
- Nurse-led discharge pathway where possible
- Constant audit of service

Conclusion

- Gynecological operations range from minimally invasive to major surgeries.
- Indication for the surgery and decision making play a crucial role in all the surgeries.
- Good surgical practice is a must.

References

- Gynaecology by Ten Teachers 21st edition